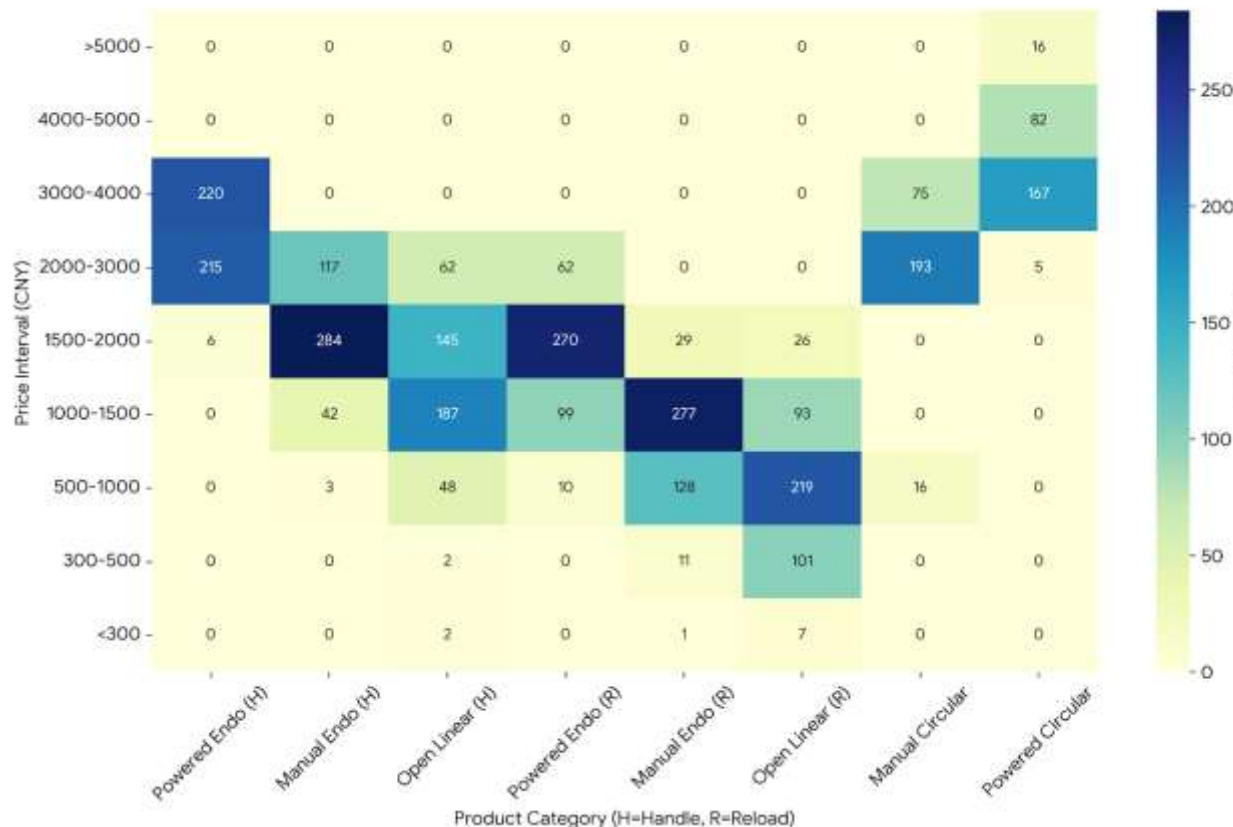


Price Sweet Spot

Surgical Stapler Price Distribution Heatmap (2025 Market Survey)



•**Powered Technology Drives Significant Premium** Powered endoscopic handles are largely concentrated in the **2,000–4,000 CNY** range, representing a **25%–50% premium** over manual products. This reflects a strong willingness to pay for enhanced clinical safety and stability.

•**Powered Circulares Set the High-End Ceiling** Powered circular staplers occupy the highest price bracket (primarily **3,000–5,000+ CNY**), representing high technical barriers and the value of innovation in complex colorectal and gastrointestinal procedures.

•**Open & Manual Categories Act as the Price Floor** Driven by Volume-Based Procurement (VBP), open linear staplers and manual reloads are heavily clustered **below 1,000 CNY**, where competition is driven strictly by cost-efficiency.

•**Reload Consumable Costs Create DRG/DIP Pressure** The single-fire cost of powered reloads (1,500–2,000 CNY) is nearly equivalent to the cost of a manual handle, making consumable usage a primary focal point for hospital cost containment. **Strategic Divide:** The **3,000 CNY** mark acts as the division between the high-end market driven by **clinical safety and technological barriers** and the volume market driven by **cost and efficiency**.

•Gastrectomy and Lobectomy:

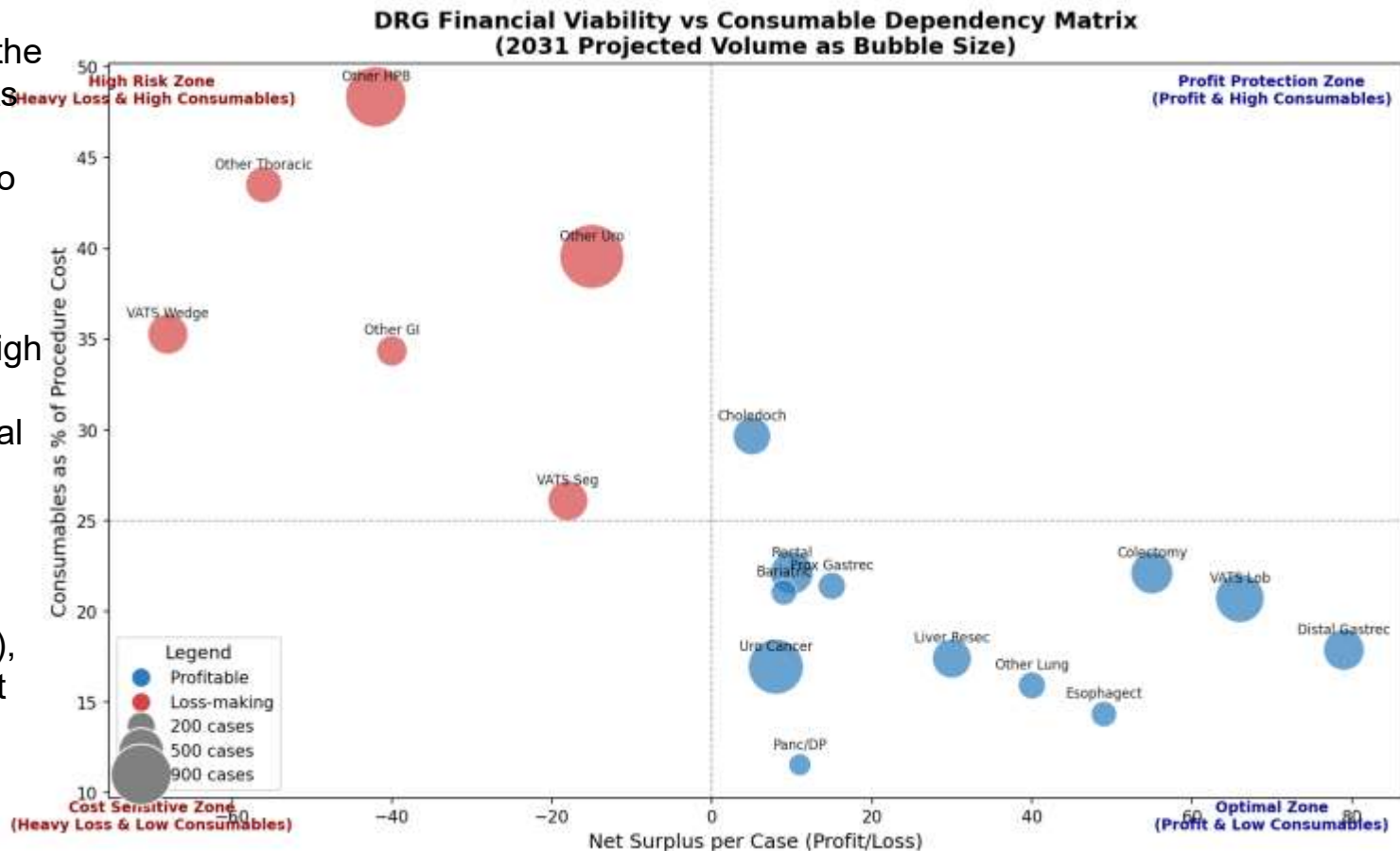
Drive the highest excess costs and volume.

Standardize these to maximize cost reduction.

•Hepatobiliary and Thoracic:

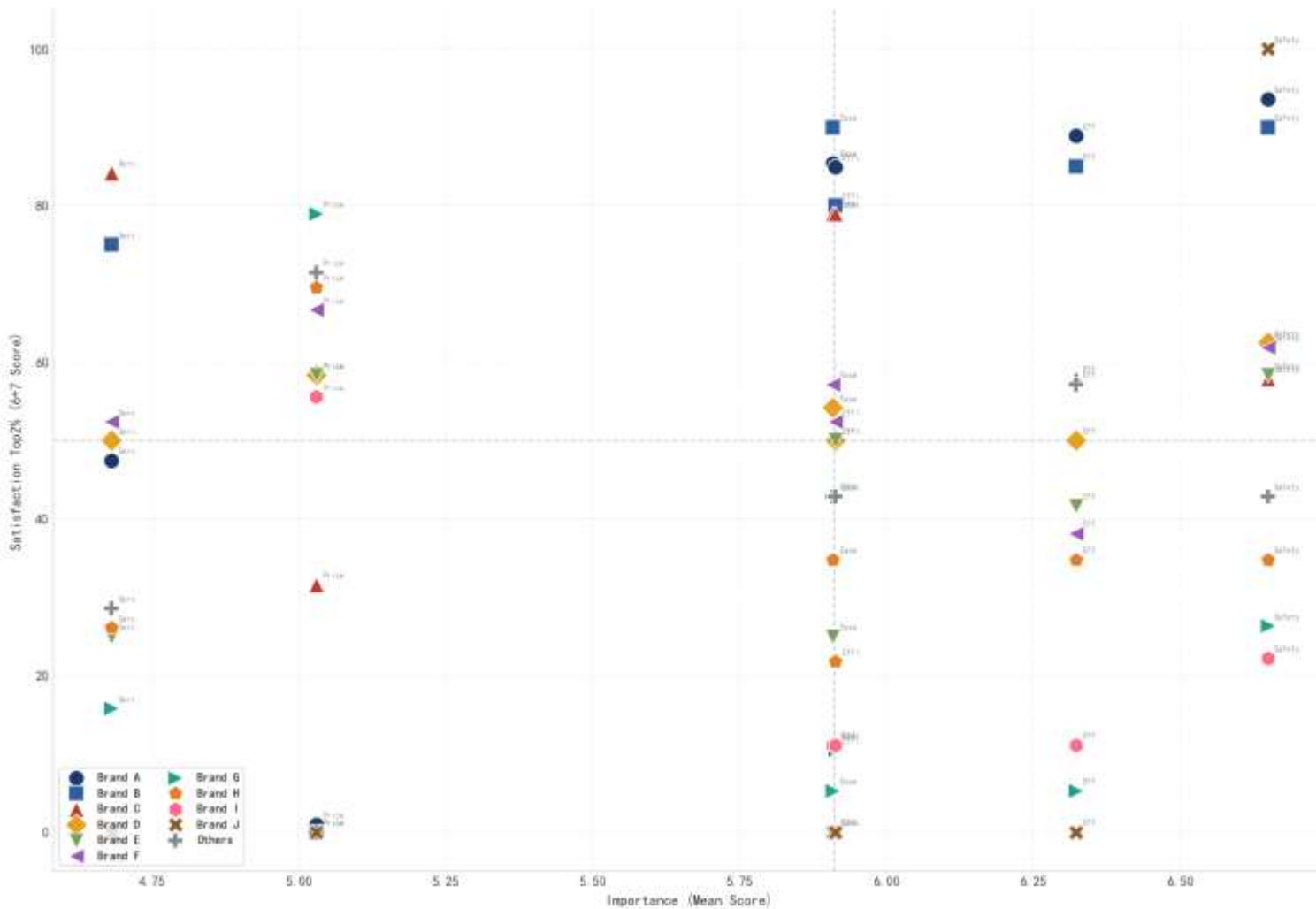
Exhibit high consumable ratios (>40%). Audit clinical practices to prevent wastage.

•**Urology:** Has the highest projected 2031 volume (1004), offering the greatest long-term ROI for efficiency gains.

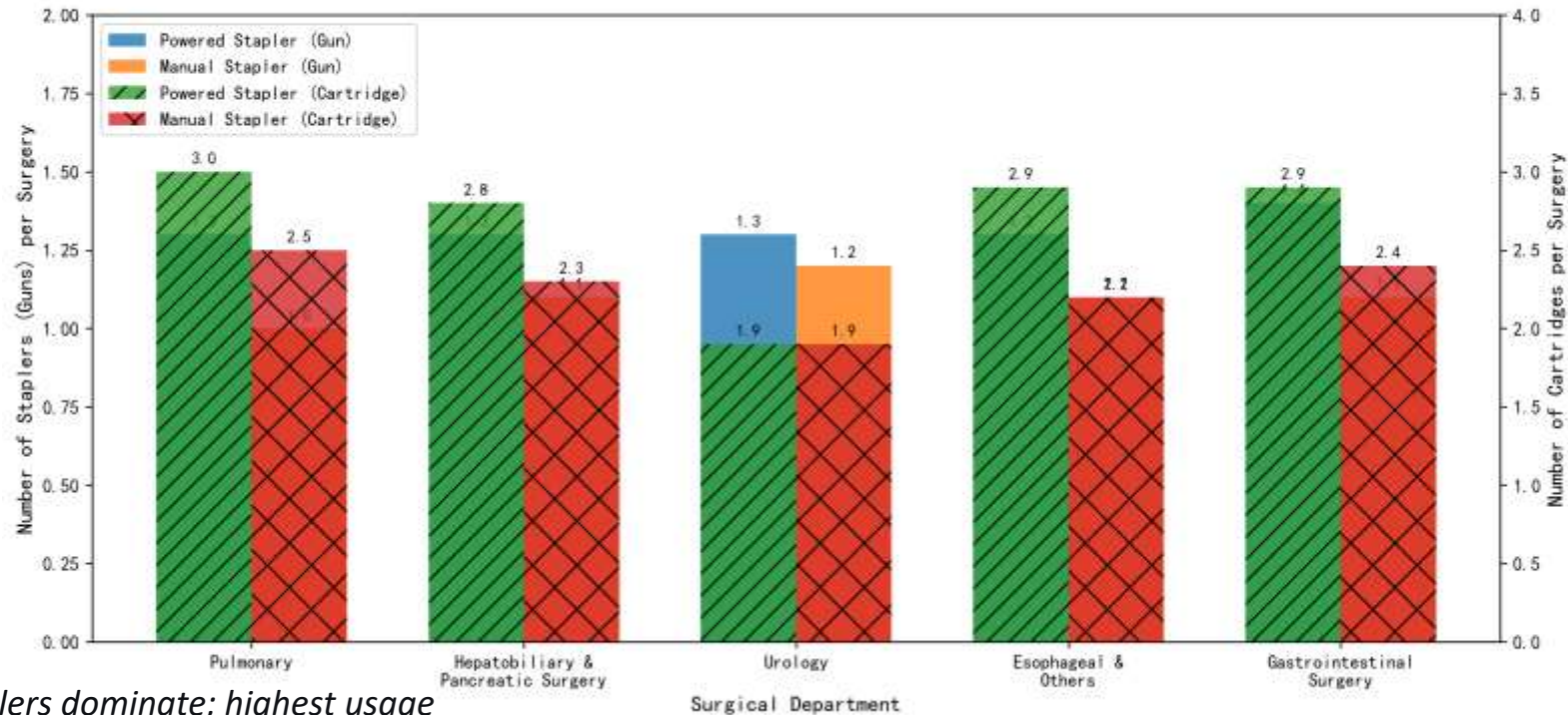


Resource Allocation: Ignore Gynecology, Thyroidectomy, and Mastectomy as they show no consumable usage.

KDA Matrix: Importance vs Brand Performance (All 11 Brands, Complete Data)



Stapler Usage per Surgery by Department (2025 Actual Data)

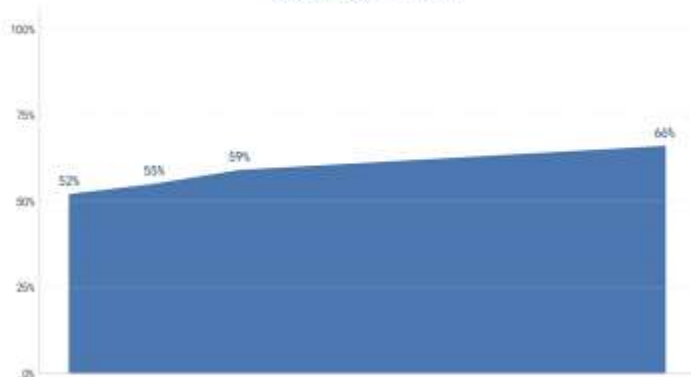


Powered staplers dominate; highest usage in Pulmonary & GI surgery. Urology uses fewest staplers – procedure-driven. Stable gun-to-cartridge ratio (~1:2.5), no major waste.

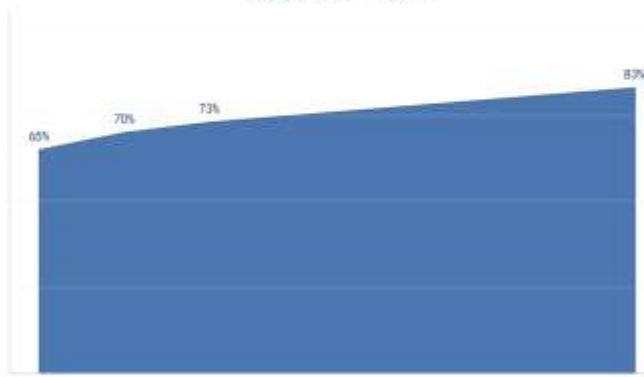
Focus on Pulmonary & GI: prioritize powered staplers & support. Explore urology: design dedicated devices + prove value. Promote manual-to-powered switch: trials & clinical data. Benchmark gun-cartridge ratio; feedback abnormal usage.

Stapler Electrification Evolution (S-Curve)
Tier A vs Tier C Hospitals in Broad & Core Markets

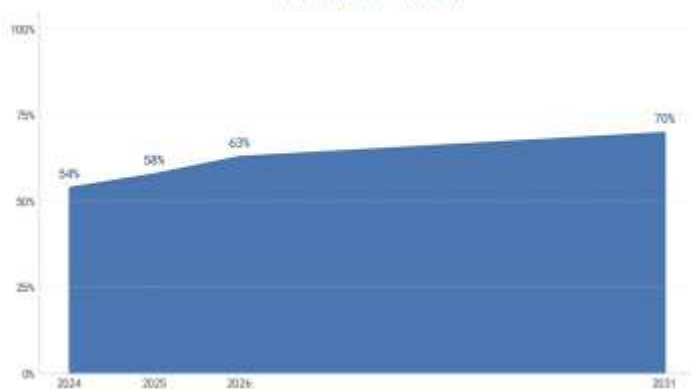
Broad Market - Tier A



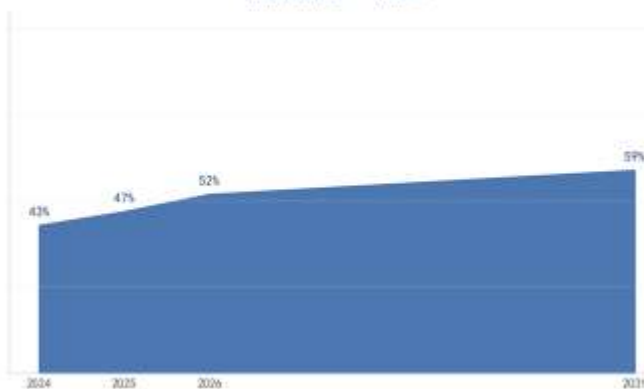
Broad Market - Tier C



Core Market - Tier A



Core Market - Tier C



1. Electric staplers will dominate (70-83% by 2031) across all markets.

→ Plan for manual stapler phase-out; shift R&D and inventory to electric platforms.

2. Tier C hospitals (Core-C, Broad-C) are closing the adoption gap fastest.

→ Launch mid-range / value-based electric products now to capture this conversion wave.

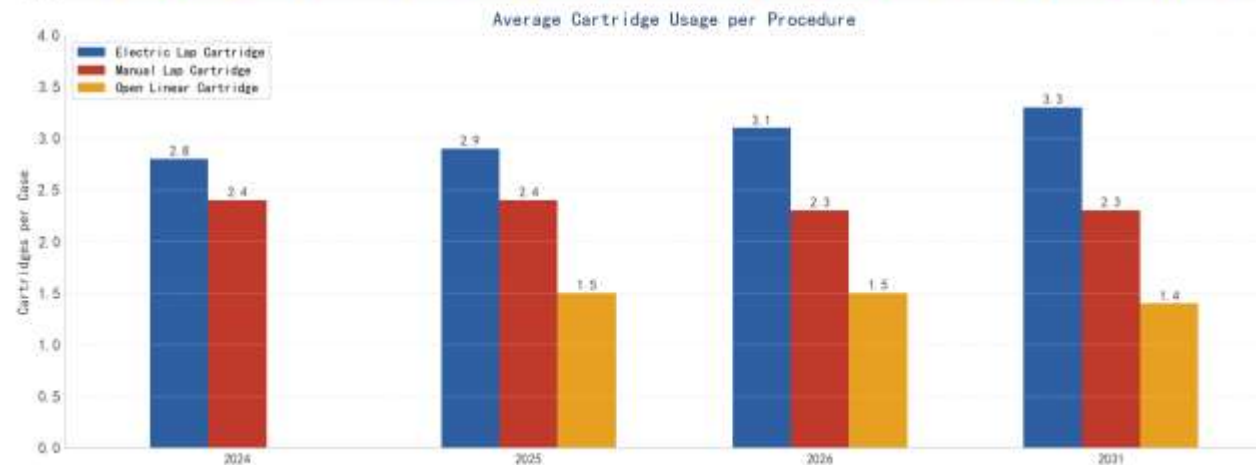
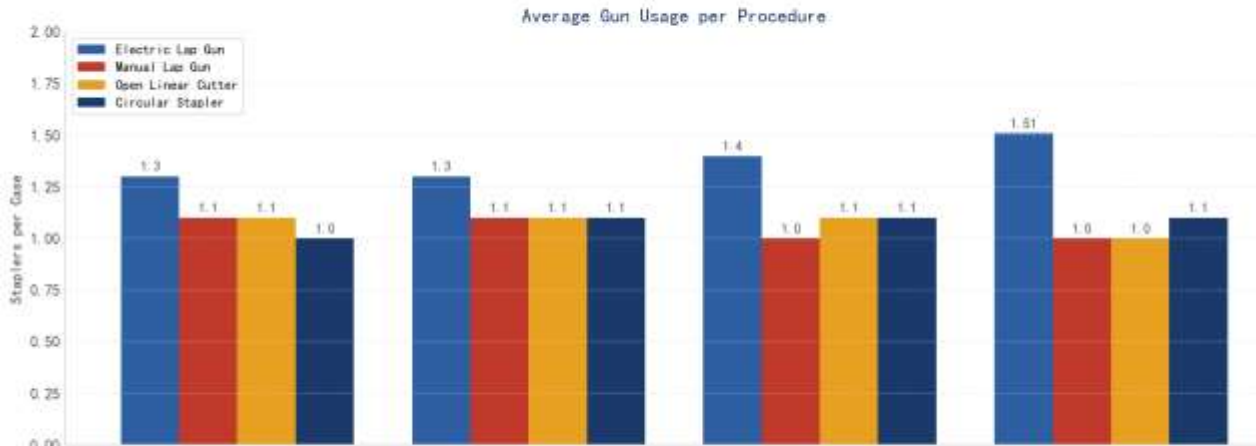
3. Broad-C shows the highest electrification target (83%).

→ Establish competitive pricing and local service support to win in this price-sensitive but high-volume segment.

4. 3-4 year technology diffusion lag between Tier A and Tier C.

→ Use premium offerings to lock Tier A loyalty today, while preparing cost-optimized solutions for Tier C tomorrow.

Management Infographics



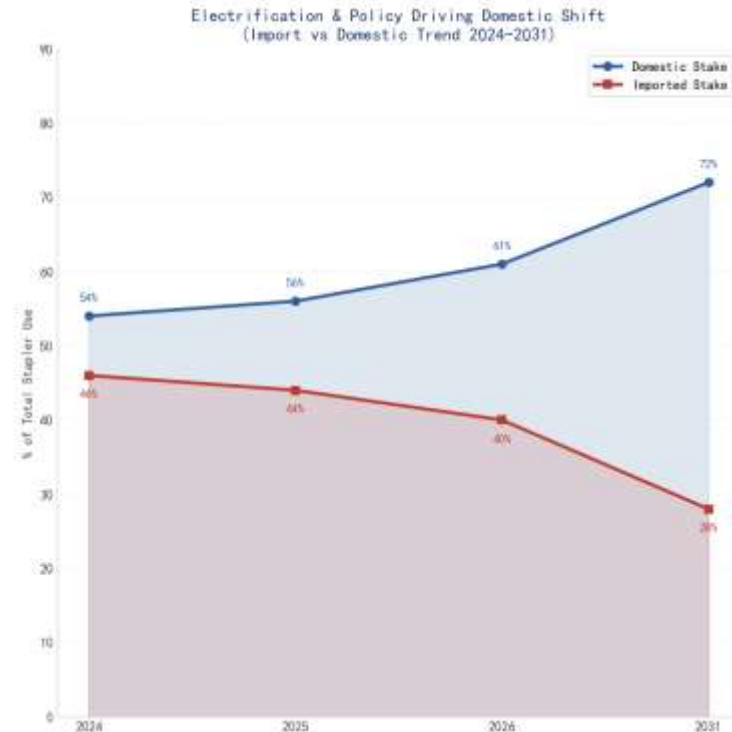
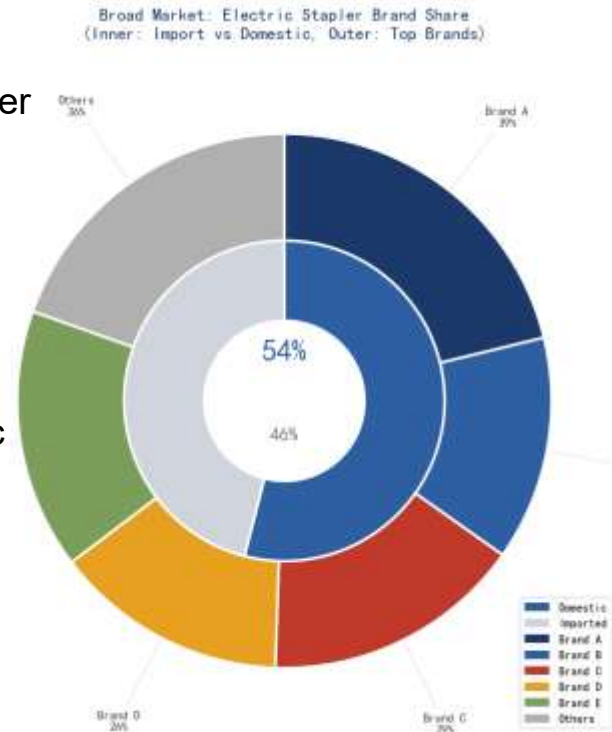
1. Electric stapler gun usage rises to 1.5 per case by 2031, while manual drops to 1.0.→ Shift sales focus to electric platforms; train clinicians for higher gun utilization per case.
2. Electric cartridge consumption increases steadily (2.8→3.3), widening gap vs. manual (2.4→2.3).→ Bundle guns with multi-cartridge packs to capture more revenue per procedure.
3. Open linear cutter and circular stapler usage stays flat at 1.0-1.1.→ Maintain these legacy products with minimal investment as market transitions to laparoscopic.
4. Overall, the electric-to-manual ratio for both guns and cartridges is expanding rapidly.
 - Accelerate electrification of your product portfolio; discontinue aging manual SKUs to free resources.

Domestic brands have already taken the lead.

Brand A remains the single strongest brand, but it's under siege.

The “Others” category is dangerously large.

The march toward domestic dominance is accelerating.



KEY INSIGHTS – KDA Analysis (All 11 Brands, Desensitized)

1. ****Safety & Effectiveness dominate the strategic imperatives.****

Both dimensions score highest on importance (>6.5). Brand A achieves exceptional Top2 scores (>85%), while Brands C–J lag significantly, especially on Effectiveness. This is the primary retention risk for domestic brands.

→ Action: Invest in clinical training and surgical workshops to close the perceived performance gap.

2. ****Price is a latent disruptor.****

Price importance is the lowest (~4.2), yet domestic brands (C–J) already score 70-100% satisfaction on Price, systematically eroding the premium pricing power of Brands A and B.

→ Action: Shift the conversation from pure price to total value – emphasize Safety + Efficiency as the real cost savers.

3. ****Service is an under-leveraged differentiator for mid-tier brands.****

Service importance is moderate, and Brand A leads, but Brands B, E, F, G show considerable room for improvement.

→ Action: Increase dedicated technical support and academic sponsorships, especially for Brands B and F, to convert passive users into loyalists.

4. ****Efficiency is the rising star.****

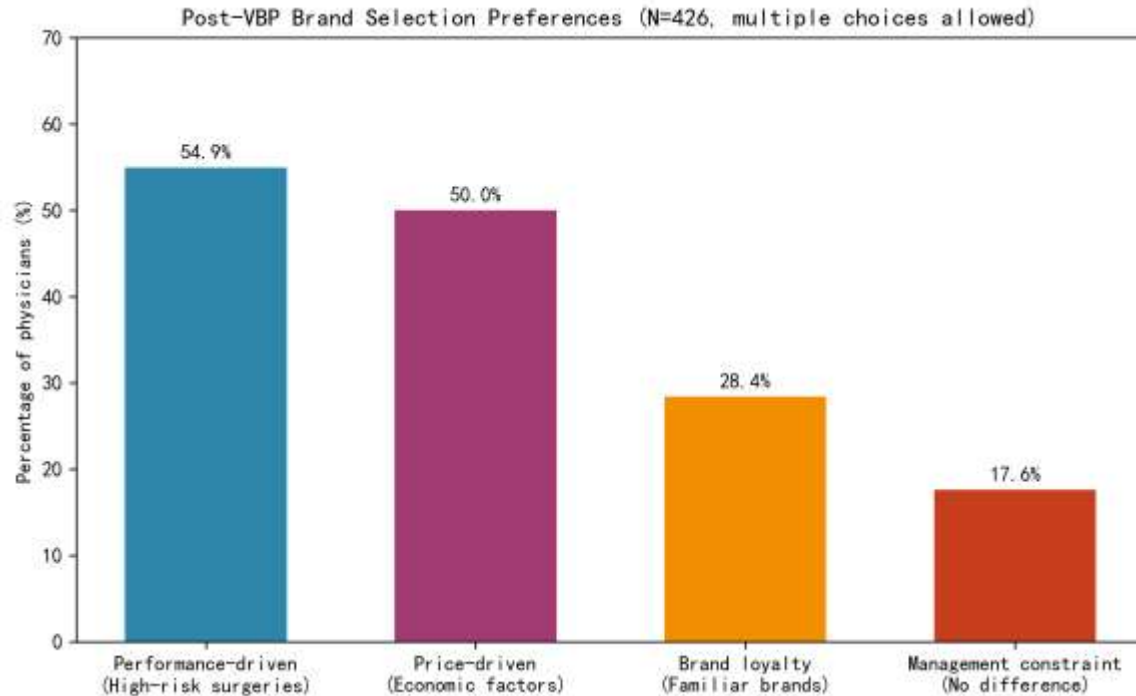
High importance and a clear performance gradient (Brand A > B > D > others) make Efficiency a decisive factor.

As surgical volumes grow, time-saving features (powered stapling) will influence brand choice more than ever.

→ Action: Embed 'Efficiency' messaging in marketing for Brands A and B; domestic competitors must accelerate R&D on powered platforms.

5. ****Smaller brands (I, J) suffer from tiny sample sizes, making their scores highly volatile.****

Interpret their positions with caution – they need much larger user bases before any strategic conclusions can be drawn.



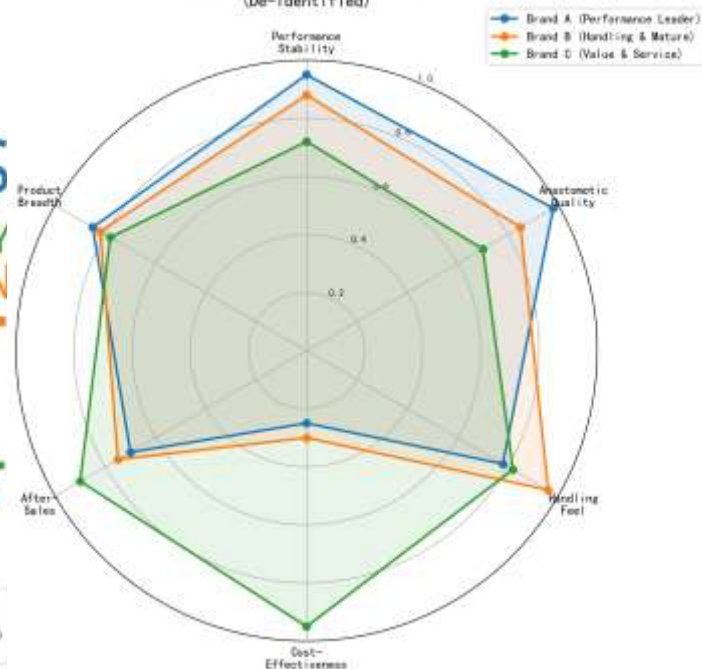
Performance → Target complex surgeries with clinical evidence.

Price → Offer value-based products for routine procedures.

Loyalty → Strengthen surgeon training and long-term relationships.

Management → Work with hospital procurement to reduce barriers.

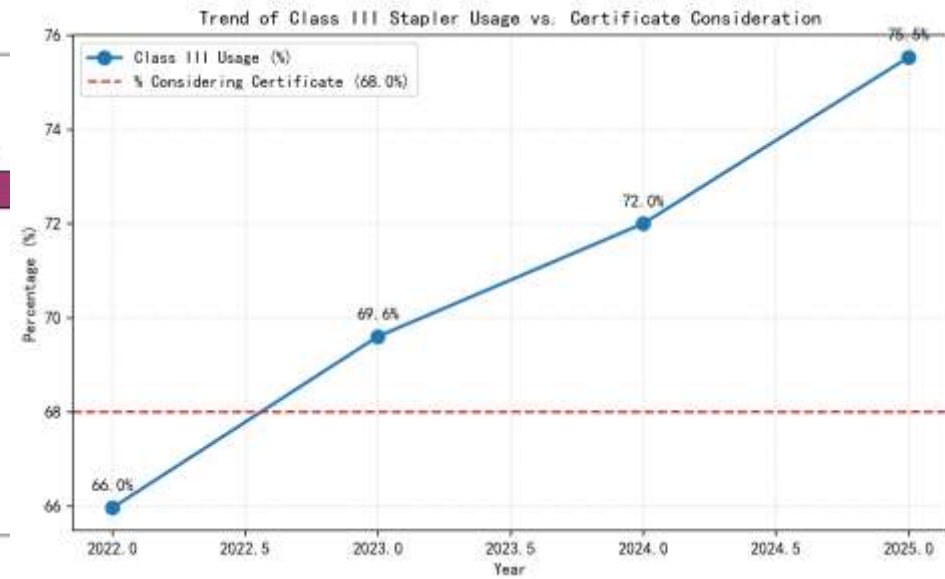
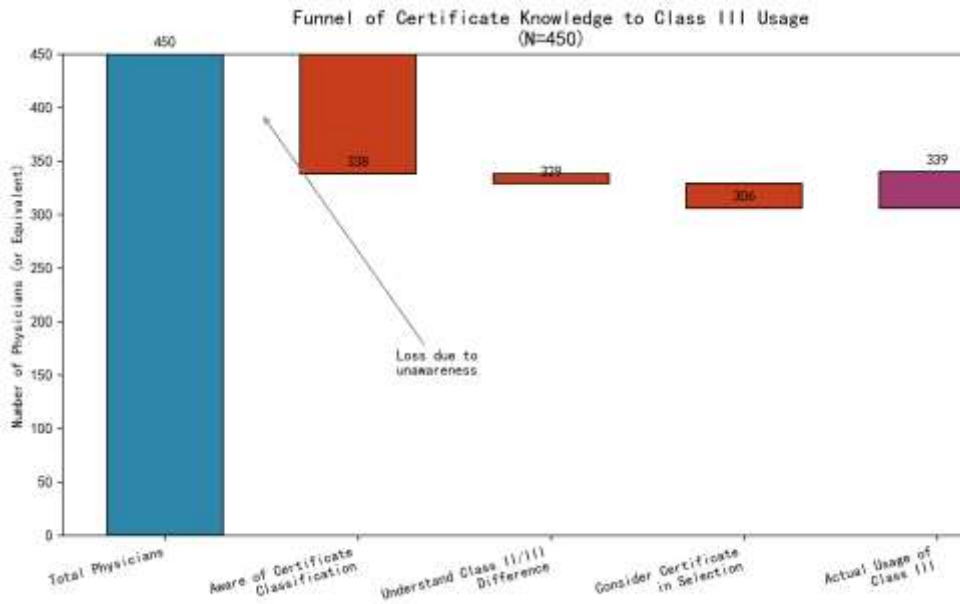
**55% prioritize performance in high-risk surgeries.
50% shift to price/value due to economic pressure.
Only 28% stay brand-loyal; 18% remain
under management control.**



● Multinational = performance for complex cases;
Domestic = value for routine surgery; Clinical feel is now table stakes.

Differentiate positioning: Brand A = complex/oncology, Brand B = general/thoracic, Brand C = primary/emergency.

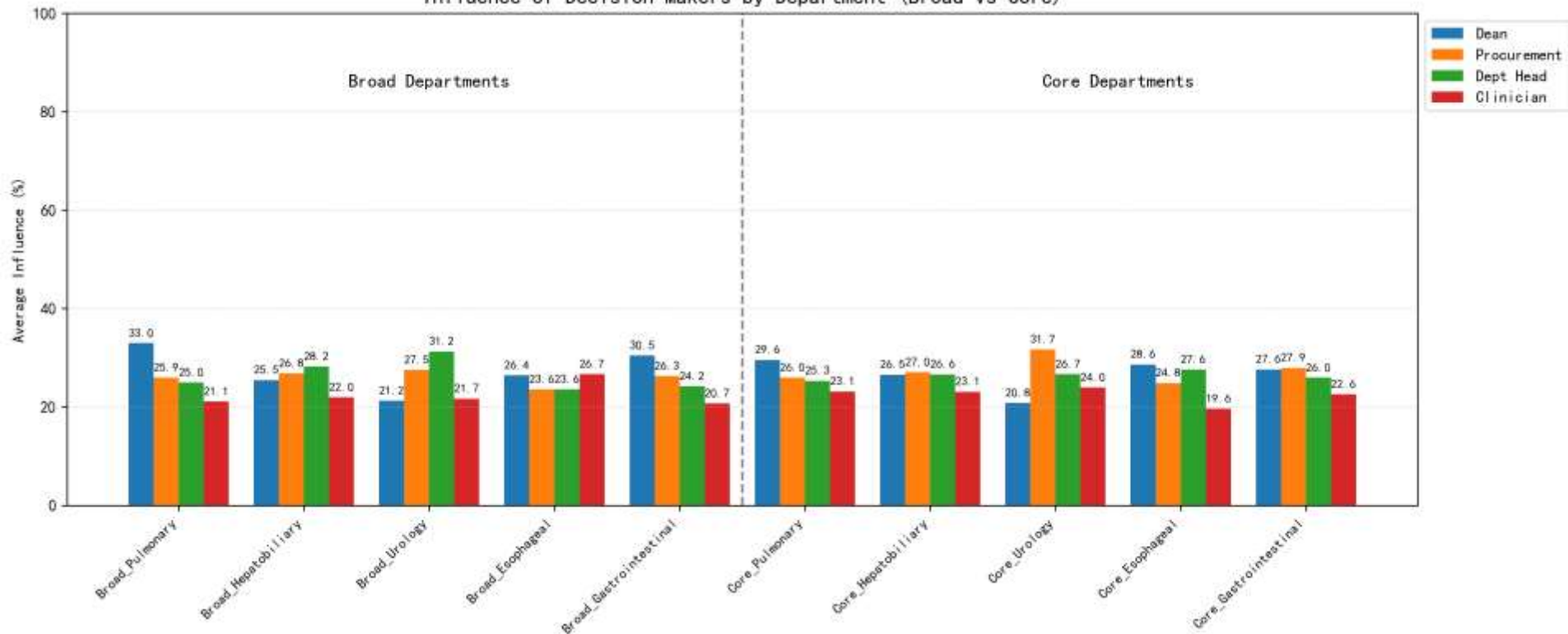
Brand A wins on performance, Brand B on handling, Brand C on value – segment marketing accordingly.



1. Awareness of certificate classification: 75.1%
2. Understanding of Class II/III difference (among aware): 97.3%
3. Consider certificate in brand selection (among aware): 90.5%
4. Average Class III usage proportion 2025: 75.5%
5. The gap between 'consideration' (68.0%) and actual Class III usage (76.2% in 2025) suggests that physicians tend to use more Class III devices than their stated consideration, possibly driven by product availability, hospital policies, or perceived clinical superiority of Class II I (often imported).

6. Class III usage has steadily increased from 66.0% in 2022 to 75.5% in 2025, indicating a shift toward higher-certification devices, which potentially serves as an 'academic fortress' for imported brands against cost-effective domestic Class II products.

Influence of Decision Makers by Department (Broad vs Core)



In Broad departments (general hospitals/less complex cases), administrative roles (Dean, Procurement) have higher influence on brand selection.

In Core departments (specialized/tertiary centers), clinical leaders (Dept Head, Attending Physician) dominate the decision-making process. The shift from “administrative-driven” to “clinician-driven” is clearly visible when moving from Broad to Core departments.

For Broad departments – Focus marketing & negotiation on hospital administration. Emphasize cost-effectiveness, supply chain reliability, and compliance with VBP policies.

For Core departments – Engage clinical leaders directly. Provide scientific evidence, peer-reviewed publications, and hands-on training to showcase clinical performance and safety.

Tailored value proposition – For Broad: bundle pricing, stable supply. For Core: advanced features (e.g., powered staplers, smart sensing), clinical outcome data.

Cross-functional alignment – In Core departments, also maintain good relationships with procurement to facilitate access, but the primary driver remains clinical endorsement.